- Chest X-ray
- CT scan
- Arterial blood gas analysis

Medical Management

- Inhaled Bronchodilators
- Anticololinergic
- Antibiotics
- Corticosteroids nebulization
- Methylxanthines
- Digitalis – to treat LVF, if present
- Mucolytics eg: potassium iodide, guaifenesin
- Immunization
- Oxygen therapy
- Chest physiotherapy

Surgical management

- Lung volume reduction surgery
- Lung transplantation

Prevention

- Avoid cigarette smoking and other inhaler irritants.
- Control of environmental temperature and humidity
- Proper nutrition and adequate hydration
- Pneumococcal vaccination

Nursing management:

- Eliminate pulmonary irritants like cigarette smoking
- Train and monitor patients inhaler using techniques
- Restrict sodium
- Encourage relaxation exercises

Complications

- Respiratory infection.
- Respiratory failure
- Right side heart failure
- Pneumonia.
- Depression



Incidence of COPD is 9/1000 per year globally with a higher incidence in males and in smokers.

2.19 Diabetes Mellitus

Diabetes mellitus is a chronic disease caused by inherited and/or acquired deficiency in production of insulin by the pancreas, or by the increased insulin resistance such a deficiency results in increased concentrations of glucose in the blood, which in turn damage many of the body's systems, in particular the blood vessels and nerves.

Definition

The term Diabetic mellitus describes a metabolic disorder of multiple etiologies characterized by chronic hyperglycemia with disturbance of carbohydrate fat and protein metabolism resulting from defects of insulin secretion, insulin action or both.

Types

- Type 1
- Type 2
- Prediabetes
- Gestational diabetes

Type 1 diabetes/Insulin dependent/ Diabetes mellitus (IDDM)

Type 1 diabetes is also referred to as Juvenile diabetes Mellitus. It results from destruction of pancreatic β cells which produce insulin leading to absolute insulin deficiency.

Etiology

- Viral
- Autoimmune
- Environmental factors.

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